Schizophrenia spectrum and other psychotic disorders

Start here when someone has delusions, hallucinations, disorganized speech, grossly disorganized or catatonic behavior, or negative symptoms. Work from the top. The first yes is usually your answer.

The whole chapter at a glance, in DSM-5-TR order

- **Schizotypal personality disorder** odd beliefs, no sustained psychosis
- **Delusional disorder** delusions only, 1 month+
- **Brief psychotic disorder** 1 day to under 1 month
- **Schizophreniform disorder** 1 to under 6 months
- **Schizophrenia** 6 months or more
- **Schizoaffective disorder** psychosis plus major mood episodes

DSM-5-TR arranges the chapter along a gradient of psychopathology: fewer symptom types and shorter illness first. It is not a ranking of outcome; schizoaffective disorder tends to have a better course than schizophrenia. Substance, medical and catatonia diagnoses sit alongside, and you rule those out first.

RULE OUT FIRST

1 Sudden onset, with attention and awareness that come and go?

Psychosis can be part of delirium, especially in older or medically ill people.

Yes →

Delirium

A medical emergency. Get medical evaluation now; psychosis here is a symptom, not a separate disorder.

No ↓

2 Caused by a substance or medication?

Symptoms began during or soon after intoxication, withdrawal or a medication that can cause psychosis (for example stimulants, cannabis, steroids, alcohol withdrawal).

Yes →

Substance/medication-induced psychotic disorder

If psychosis came before the use, or lasts a long time (about a month) after stopping, think of a primary psychotic disorder instead.

No ↓

3 Caused by another medical condition?

History, exam or tests show a condition that explains it (for example a brain tumor, seizures, lupus, thyroid disease).

Yes →

Psychotic disorder due to another medical condition

Red flags: first episode after about age 40, visual, tactile or smell hallucinations, head injury, new neurological signs.

No ↓

MOOD OR PSYCHOSIS?

4 Does the psychosis happen only during depressive or manic episodes?

When the mood episode clears, the delusions and hallucinations clear too.

Yes →

Major depressive disorder, Bipolar I or Bipolar II, with psychotic features

Psychosis during a high always means mania (Bipolar I). Psychosis during a depression can occur in MDD, Bipolar I or Bipolar II.

No ↓

5 A full schizophrenia picture during a major mood episode, mood episodes for most of the illness, AND 2 weeks or more of delusions or hallucinations with no mood episode?

All three are needed (a depressive episode must include depressed mood). Delusions alone plus long mood episodes is other specified, not schizoaffective.

Yes →

Schizoaffective disorder

Bipolar type if a manic episode is part of it; depressive type if only major depressive episodes.

No ↓

HOW MUCH PSYCHOSIS?

6 Only delusions, for 1 month or more, and the person otherwise functions fairly well?

No other prominent psychotic symptoms. Any hallucinations are minor and fit the delusional theme. Behavior is not obviously odd outside the delusion.

Yes →

Delusional disorder

Types: erotomanic, grandiose, jealous, persecutory, somatic, mixed, unspecified; specify if with bizarre content. A fixed false belief about appearance is body dysmorphic disorder with absent insight instead.

No ↓

Otherwise: how long has it lasted?

For schizophreniform disorder and schizophrenia, at least 2 of the 5 symptom types are needed, and one must be delusions, hallucinations or disorganized speech. Brief psychotic disorder needs only 1 of those 3. If only one symptom type lasts a month or more and delusional disorder doesn't fit, use other specified.

1 DAY TO UNDER 1 MONTH

Brief psychotic disorder

Full return to usual functioning. Specify with or without a marked stressor, or onset during pregnancy or within 4 weeks after delivery.

1 MONTH TO UNDER 6 MONTHS

Schizophreniform disorder

Call it provisional while the person is still ill. A drop in functioning is not required. Specify with or without good prognostic features (rapid onset, confusion at the peak, good prior functioning, no flat affect).

6 MONTHS OR MORE

Schizophrenia

Counts warning-sign and leftover periods, with at least 1 month of active symptoms (less if successfully treated). Functioning has dropped, or in child or teen onset never reached the expected level.

Also in this chapter

Schizotypal personality disorder

A lifelong pattern of odd beliefs, eccentric behavior and social anxiety, without sustained psychosis. Listed here and with the personality disorders.

Catatonia

3 or more of 12 motor features, such as stupor, waxy flexibility, mutism, negativism, posturing or echoing. Recorded with another mental disorder, due to a medical condition, or unspecified.

Other specified or unspecified

Psychosis that doesn't fit, such as persistent voices with no other symptoms. 'Unspecified' when there isn't enough information yet, as in an emergency room.

Culture check

Shared beliefs are not delusions

Beliefs or experiences widely shared in the person's cultural or religious group (for example witchcraft, spirit possession, religious visions) are not delusions or hallucinations.

Grief experiences are normal

Briefly hearing or seeing a loved one who has died is a common part of bereavement, not psychosis.

Exam traps

- Naming schizophrenia before ruling out substances and medical causes.
- Calling 4 months of psychosis schizophrenia. Under 6 months is schizophreniform.
- With autism or a childhood communication disorder in the history, add schizophrenia only if prominent delusions or hallucinations last 1 month or more.
- Choosing schizoaffective when psychosis appears only during mood episodes.
- Diagnosing a primary psychotic disorder in an older adult with new visual hallucinations before medical review.
- Racialized groups are overdiagnosed with schizophrenia; weigh mood symptoms and culture carefully.

DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders · Original study aid; criteria paraphrased.

Bipolar and depressive disorders

Start with any mood problem. Ask about the highest high before the lowest low: one past manic episode changes the whole diagnosis.

RULE OUT FIRST

1 Caused by a substance or medication?
Started with intoxication, withdrawal or a new medication (for example steroids, stimulants, isotretinoin, interferon).

Yes →

Substance/medication-induced depressive or bipolar disorder
A full high that starts on an antidepressant and outlasts the drug's effect counts as a real episode: think bipolar.

No ↓

2 Caused by another medical condition?
For example hypothyroidism, Cushing's, stroke, Parkinson's disease, multiple sclerosis, cancer.

Yes →

Depressive or bipolar disorder due to another medical condition
Mood change that tracks the illness. Refer for a medical workup before naming a primary mood disorder.

No ↓

HIGHS

3 Ever a manic episode?
A clear stretch of unusually high, expansive or irritable mood plus extra energy, with 3 or more manic symptoms (4 if the mood is only irritable), for 1 week or more (any length if hospitalized), with marked impairment or psychosis.

Yes →

Bipolar I disorder
One manic episode is enough. Depression is common but not required.

No ↓

4 Ever a hypomanic episode AND ever a major depressive episode?
Hypomania: 4 days or more, a clear change others notice, but no marked impairment, no hospitalization and no psychosis.

Yes →

Bipolar II disorder
Never a full manic episode. Bipolar II is not a milder Bipolar I; the depressions are often long and disabling. Hypomania without any major depressive episode is other specified bipolar and related disorder.

No ↓

CHRONIC PATTERNS

5 For 2 years (1 year in youth), many highs and lows that never reach full episodes?
Hypomanic-type and depressive-type symptoms present at least half the time, never symptom-free for more than 2 months at a time.

Yes →

Cyclothymic disorder
If a full manic, hypomanic or major depressive episode ever occurs, change the diagnosis.

No ↓

SPECIFIC PICTURES

6 A child (diagnosed ages 6 to 18, onset before 10) with severe outbursts 3 or more times a week and irritable mood between them, for 12 months?
Chronic, not episodic. Present in at least two settings, with no symptom-free stretch of 3 months or more.

Yes →

Disruptive mood dysregulation disorder
Created to avoid overdiagnosing bipolar disorder in children. It overrides ODD and intermittent explosive disorder, but can coexist with MDD, ADHD and conduct disorder.

No ↓

7 **5 or more symptoms (at least 1 mood symptom) in the week before menstruation, improving within a few days after it starts and minimal the week after, in most cycles?**

Mood swings, irritability, depressed mood or anxiety, plus symptoms such as bloating or breast tenderness. Not just a premenstrual worsening of MDD or PDD.

Yes →

Premenstrual dysphoric disorder

Confirm with daily ratings over at least 2 cycles.

No ↓

LOWS

8 **Depressed mood on most days for 2 years or more (1 year in youth)?**

Plus 2 or more of: appetite, sleep, energy, self-esteem, concentration, hopelessness. Mood can be irritable in youth. Never free of symptoms for more than 2 months.

Yes →

Persistent depressive disorder

If major depressive episodes also occur, record both (PDD with intermittent or persistent major depressive episodes).

No ↓

9 **5 or more of 9 symptoms for 2 weeks, including depressed mood or loss of interest?**

Sleep, interest, guilt or worthlessness, energy, concentration, appetite or weight, slowing or agitation, thoughts of death.

Yes →

Major depressive disorder

Single or recurrent. Grief does not rule it out: look for worthlessness and a pervasive low mood rather than waves of longing.

Episode clocks

The minimum time for each kind of episode or pattern.

4 DAYS	**Hypomanic episode**	No marked impairment, no psychosis.
1 WEEK	**Manic episode**	Or any length if hospitalized.
2 WEEKS	**Major depressive episode**	5 of 9 symptoms, nearly every day.
2 YEARS (1 IN YOUTH)	**Persistent depressive and cyclothymic disorders**	No symptom-free stretch longer than 2 months.

Specifiers worth knowing

With peripartum onset

Mood episode begins during pregnancy or within 4 weeks after delivery.

With seasonal pattern

Episodes start and end at the same times of year, for at least 2 years.

With rapid cycling

Bipolar disorder with 4 or more mood episodes in 12 months.

With mixed features or anxious distress

Opposite-pole symptoms during an episode, or marked anxiety; both raise risk.

Exam traps

- Calling 3 days of elevated mood hypomania. It needs 4.
- Diagnosing bipolar disorder in a child with chronic, non-episodic irritability: think DMDD.
- Starting with depression before asking about past highs. Antidepressants alone can trigger mania in bipolar disorder: refer to the prescriber.
- Psychosis during a high means mania, so Bipolar I, never Bipolar II.
- Assuming grief rules out major depression. It doesn't.

DSM-5-TR (APA, 2022), Bipolar and Related Disorders; Depressive Disorders · Original study aid; criteria paraphrased.

Trauma- and stressor-related disorders

Every diagnosis in this chapter needs a stressor. Ask what happened, how old the person is, and how long ago it was.

YOUNG CHILDREN WITH SEVERE NEGLECT

1 **A child with a history of grossly inadequate care who rarely seeks or responds to comfort?**

Withdrawn and emotionally flat with caregivers. Developmental age at least 9 months; signs before age 5. If the child re-enacts a specific frightening event in play or has nightmares about it, check PTSD (age 6 and under criteria) first.

Yes →

Reactive attachment disorder

Inhibited. Rule out autism, which has restricted, repetitive behavior and no neglect requirement.

No ↓

2 **Same history of inadequate care, but the child is overly friendly with strangers?**

Wanders off with unfamiliar adults and doesn't check back with the caregiver. Developmental age at least 9 months.

Yes →

Disinhibited social engagement disorder

Disinhibited. It can persist even after the child is placed in a loving home.

No ↓

DEATH OF SOMEONE CLOSE

3 **A death 12 months or more ago (6 months in a child), with yearning or preoccupation plus 3 or more other grief symptoms, nearly every day for the past month, beyond cultural norms?**

For example identity disruption, disbelief, avoiding reminders, intense pain, numbness, meaninglessness, loneliness.

Yes →

Prolonged grief disorder

New in DSM-5-TR. Before 12 months, intense grief is usually expected; check for major depression instead. A violent or accidental death also counts as trauma exposure; check PTSD too.

No ↓

TRAUMATIC EVENT

4 **Did the person face death, serious injury or sexual violence, or a real threat of them?**

Directly, as a witness, learning it happened to someone close (violent or accidental), or repeated exposure to details at work. Media exposure alone doesn't count unless it's work-related.

Yes →

Go to the trauma clock below

Intrusions, avoidance, negative mood and thinking, and arousal.

No ↓

ANY OTHER STRESSOR

5 **Distress or impairment starting within 3 months of any stressor, not meeting another disorder?**

Job loss, divorce, a move, illness. Also a trauma when ASD or PTSD criteria aren't met.

Yes →

Adjustment disorder

A leftover category: if MDD, PTSD or another disorder fits, use that. Symptoms end within 6 months after the stressor and its effects end.

No ↓

6 **A reaction in line with what's expected and culturally normal?**

For example grief that comes in waves, with self-esteem intact.

Yes →

No mental disorder (normal or expected reaction)

Support, psychoeducation and watchful follow-up, not a diagnosis.

Trauma clock: time since the event

Measure from the traumatic event to now, and check the full criteria at each point. If full ASD or PTSD criteria aren't met at that point, go back to adjustment disorder (or other specified trauma- and stressor-related disorder).

UNDER 3 DAYS	3 DAYS TO 1 MONTH	MORE THAN 1 MONTH	FULL CRITERIA 6+ MONTHS LATER
Expected acute reaction Too soon for a diagnosis.	**Acute stress disorder** 9 or more of 14 symptoms across intrusion, mood, dissociation, avoidance and arousal.	**Posttraumatic stress disorder** At least 1 intrusion, 1 avoidance, 2 negative mood or thinking, and 2 arousal symptoms. Separate criteria for age 6 and under.	**PTSD with delayed expression** Some symptoms may start early; the full picture comes later.

Exam traps

- Calling it PTSD in the first month. Under 1 month is acute stress disorder.
- Using adjustment disorder when full MDD or PTSD criteria are met.
- Diagnosing prolonged grief disorder before 12 months in an adult.
- Counting trauma seen only on TV or social media (unless it's work-related).
- Mixing up RAD (withdrawn) and DSED (too friendly).

DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders · Original study aid; criteria paraphrased.

Anxiety disorders (and OCD)

Ask what the fear is about and how long it has lasted. Most anxiety disorders need about 6 months in adults.

RULE OUT FIRST

1 **Caused by a substance, withdrawal or a medical condition?**
Caffeine, stimulants, alcohol or sedative withdrawal; hyperthyroidism, heart rhythm problems, asthma, low blood sugar.

Yes → **Substance/medication-induced or medical anxiety disorder**
First panic attack after 45, or with chest pain, fainting or confusion: get a medical evaluation.

No ↓

WHAT IS THE FEAR ABOUT?

2 **Being apart from attachment figures, beyond what's typical for age?**
3 or more of 8 signs, for example worry that harm will come to them, refusing to sleep away, nightmares about separation, physical complaints.

Yes → **Separation anxiety disorder**
At least 4 weeks in youth, typically 6 months in adults. It can be diagnosed in adults.

No ↓

3 **A child who consistently doesn't speak in certain settings (such as school) but speaks elsewhere?**
For at least 1 month, not just the first month of school.

Yes → **Selective mutism**
Not if the child is still learning the language or lacks comfort with it.

No ↓

4 **One specific object or situation, such as animals, heights, blood or flying?**
Fear right away, every time, avoided or endured with intense distress, for 6 months. If 2 or more agoraphobia situations are feared because escape or help might not be available, go to agoraphobia instead.

Yes → **Specific phobia**
Blood-injection-injury type can cause fainting.

No ↓

5 **Being watched, judged or embarrassed in social situations?**
Conversations, eating in front of others, performing. In children it must happen with peers, not just adults. Lasting 6 months.

Yes → **Social anxiety disorder**
'Performance only' specifier if it's limited to speaking or performing in public. Panic attacks can occur here too.

No ↓

6 **Unexpected panic attacks, then 1 month or more of worry about more attacks or a change in behavior?**
Attacks peak within minutes and come out of the blue, at least at first.

Yes → **Panic disorder**
Panic attacks that only happen in feared situations belong to that disorder (for example social anxiety), not panic disorder. If they also avoid 2 or more agoraphobia situations for 6 months, add agoraphobia as a second diagnosis.

No ↓

7 **Fear of 2 or more of: public transport, open spaces, enclosed places, crowds or lines, being outside home alone?**
Because escape or help might not be available. Lasting 6 months.

Yes → **Agoraphobia**
Diagnosed on its own, with or without panic disorder.

No ↓

8 **Hard-to-control worry about many things, most days, for 6 months, with 3 or more of 6 associated symptoms (1 in children)?**

Restlessness, fatigue, poor concentration, irritability, muscle tension, sleep problems.

Yes →

Generalized anxiety disorder

Worry about everyday matters, not one feared thing.

No ↓

RELATED CHAPTERS

9 **Intrusive unwanted thoughts, repetitive acts done to reduce the distress, or both?**

Usually more than an hour a day, or clearly impairing.

Yes →

Obsessive-compulsive disorder

In its own chapter. Specify insight (good or fair, poor, or absent with delusional beliefs, which is still OCD, not psychosis) and whether it is tic-related.

Exam traps

- Missing a medical cause for a first panic attack in an older adult.
- Calling panic attacks that occur only when speaking in meetings panic disorder: it's social anxiety.
- Diagnosing selective mutism in a child who is still learning the language.
- Confusing GAD (many worries) with illness anxiety (fear of having a serious disease).
- Calling OCD with absent insight a psychotic disorder.

DSM-5-TR (APA, 2022), Anxiety Disorders; Obsessive-Compulsive and Related Disorders · Original study aid; criteria paraphrased.

Somatic symptoms, factitious disorder and malingering

Two questions sort these: are the symptoms produced on purpose, and what is the person trying to get?

	Not on purpose	On purpose
No outside reward (or the goal is to be seen as sick)	**Somatic symptom disorder, Illness anxiety disorder, Functional neurological symptom disorder** Symptoms are genuinely experienced. The problem is the distress, worry or neurological symptoms, not deception.	**Factitious disorder** Fakes, causes or exaggerates illness to be seen as sick or injured, even with no reward. Imposed on self, or on another (a child, an older adult, a pet).
Clear outside reward (money, avoiding work, court or military, getting drugs)	**Still a somatic disorder** Someone may gain something by being ill without making the symptoms up. Gain alone doesn't prove faking.	**Malingering** Not a mental disorder. Suspect it with a legal or financial context, a big gap between claimed problems and objective findings, poor cooperation, or antisocial personality disorder.

Not on purpose

Real experiences, not faking

Somatic symptom disorder

Distressing body symptoms plus excessive thoughts, worry or time spent on them. Usually 6 months or more. Symptoms may or may not have a medical explanation.

Illness anxiety disorder

Few or no symptoms, but strong fear of having or getting a serious illness, for 6 months. Care-seeking or care-avoiding type.

Functional neurological symptom disorder

Weakness, seizure-like episodes, numbness or loss of speech that don't fit known neurological disease (for example a positive Hoover sign). A stressor is not required.

Psychological factors affecting other medical conditions

Behavior or emotions worsen a real medical illness, such as skipping insulin because of anxiety.

On purpose

Deception is present

Factitious disorder imposed on self

Motive is internal: the sick role. Often health care knowledge, many hospitals, eager for procedures.

Factitious disorder imposed on another

The diagnosis goes to the person doing the deceiving, not the victim. This is maltreatment: report it.

Malingering

Motive is external. Recorded as a condition that may be a focus of clinical attention, not a mental disorder.

Exam traps

- Calling functional neurological symptoms 'faking'. They are not deliberate.
- Assuming a stressor is required for functional neurological symptom disorder. It isn't in DSM-5-TR.
- Diagnosing the child instead of the caregiver in factitious disorder imposed on another.
- Mixing up factitious (internal motive) and malingering (external reward).

DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders; Other Conditions That May Be a Focus of Clinical Attention · Original study aid; criteria paraphrased.

Delirium and neurocognitive disorders

For confusion or memory problems, ask two things first: how fast did it start, and can the person still manage complex daily tasks on their own?

FAST OR SLOW?

1 **Onset over hours to days, with attention and awareness that fluctuate during the day?**

Often worse at night. Look for infection, medications, dehydration, surgery, withdrawal.

Yes →

Delirium

A medical emergency, and usually reversible. Notify the medical team first. It can sit on top of dementia.

No ↓

DEPRESSION LOOK-ALIKE

2 **Low mood and slowed thinking, with inconsistent, "I don't know" answers on testing, starting with a depressive episode?**

The person often complains loudly about memory; testing improves with encouragement.

Yes →

Major depressive disorder

Treat the depression and re-test. If problems persist after mood improves, continue to the steps below; depression can also be an early sign of a neurocognitive disorder.

No ↓

HOW MUCH?

3 **A significant decline that means the person needs help with complex tasks such as bills or medications?**

Decline from a previous level in memory, attention, language, executive, perceptual-motor or social thinking.

Yes →

Major neurocognitive disorder

Then name the cause (below). Severity: mild, moderate or severe.

No ↓

4 **A modest decline, but still independent (with more effort, lists or reminders)?**

Others may notice. Testing is below expected.

Yes →

Mild neurocognitive disorder

Watch for progression. Independence in daily activities is what separates mild from major.

Which type? Clues to the cause

Alzheimer's disease

Gradual, steady decline; memory and learning go first.

Vascular

Stepwise or sudden decline linked to strokes; slowed thinking and executive problems.

Lewy bodies

Fluctuating attention, detailed visual hallucinations, acting out dreams, parkinsonism that begins after (or within a year of) the decline, severe sensitivity to antipsychotics.

Parkinson's disease

Movement symptoms come first; cognitive decline follows later (by a year or more).

Frontotemporal

Usually begins in the 50s (range 20s to 80s); personality, behavior or language change first, memory relatively spared early.

Traumatic brain injury

Starts right after a head injury with loss of consciousness, amnesia or confusion.

Substance or medication

For example long-term heavy alcohol use; persists beyond intoxication and withdrawal.

Others

HIV, prion disease (very rapid), Huntington's disease (movements and family history).

Exam traps

- Calling sudden confusion dementia. Rule out delirium first.
- Deciding mild vs major by test scores alone. Independence in daily tasks decides it.
- Treating new visual hallucinations in an older adult as schizophrenia.
- Starting with placement or capacity talk before a medical workup.

DSM-5-TR (APA, 2022), Neurocognitive Disorders; Inouye, S. K., Westendorp, R. G. J., & Saczynski, J. S. (2014). Delirium in elderly people. The Lancet, 383(9920), 911-922, whole article · Original study aid; criteria paraphrased.

Personality disorders

Lasting, inflexible patterns that start by adolescence or early adulthood and cause distress or impairment. Group them by cluster first.

Cluster A: odd or eccentric

Paranoid

Distrust and suspicion; reads hidden threats into innocent remarks; bears grudges.

Schizoid

Detached; doesn't want close relationships; little pleasure; flat emotion.

Schizotypal

Odd beliefs or magical thinking, unusual perceptions, eccentric behavior, social anxiety that doesn't ease with familiarity.

Cluster B: dramatic, emotional or erratic

Antisocial

Disregards and violates others' rights; deceit, impulsivity, no remorse. Must be 18 or older, with conduct disorder before 15.

Borderline

Unstable relationships, self-image and mood; fear of abandonment; impulsivity; self-harm; emptiness; intense anger.

Histrionic

Attention-seeking; dramatic, shallow, quickly shifting emotion; may be seductive; thinks relationships are closer than they are.

Narcissistic

Grandiosity, need for admiration, entitlement, lack of empathy; fragile self-esteem underneath.

Cluster C: anxious or fearful

Avoidant

Wants relationships but fears rejection; feels inadequate; avoids unless sure of being liked.

Dependent

Needs to be taken care of; can't decide alone; clings; urgently seeks a new relationship when one ends.

Obsessive-compulsive (OCPD)

Perfectionism, order and control that get in the way of finishing things; rigid; sees it as the right way.

Look-alikes

Schizoid vs avoidant

Schizoid doesn't want closeness. Avoidant wants it but fears rejection.

OCPD vs OCD

OCPD: a lifelong style the person agrees with. OCD: unwanted obsessions and compulsions the person resists.

Borderline vs bipolar II

Borderline mood shifts last hours and follow interpersonal triggers. Hypomania lasts days, with less need for sleep.

Antisocial vs conduct disorder

Under 18: conduct disorder. Antisocial requires age 18 or older.

Paranoid PD vs delusional disorder

Paranoid PD is pervasive suspicion without fixed false beliefs that meet delusion level.

Schizotypal vs schizophrenia

Schizotypal has no sustained psychosis.

Rules

Don't diagnose during an acute episode

Judge personality from the long-term pattern, not a crisis or a mood episode.

Under 18

Possible only if features have lasted at least 1 year, and never antisocial personality disorder.

Culture

Behavior that fits the person's cultural norms is not a personality disorder.

Exam traps

- Diagnosing antisocial personality disorder in a 16-year-old.
- Calling borderline mood shifts bipolar disorder.
- Confusing schizoid (no wish for relationships) with avoidant (wish, but fear).
- Diagnosing a personality disorder from one session or during a mood episode.

DSM-5-TR (APA, 2022), Personality Disorders · Original study aid; criteria paraphrased.

Durations ruler: how long is long enough?

The time rules the exam loves, shortest to longest. Pair this with the Timeline game.

1 day

- Brief psychotic disorder: at least 1 day
- DMDD ruled out by manic or hypomanic symptoms over 1 day

3 days

- Acute stress disorder: 3 days to 1 month

4 days

- Hypomania: 4 days

1 week

- Mania: 1 week (any length if hospitalized)

2 weeks

- Schizoaffective: 2 weeks of psychosis without mood episode
- Major depressive episode: 2 weeks

1 month

- Brief psychotic disorder: under 1 month, full recovery
- Schizophreniform: at least 1 month (under 6)
- Schizophrenia: 1 month of active symptoms
- Delusional disorder: 1 month
- PTSD: more than 1 month
- Panic disorder: 1 month of worry or change

3 months

- Adjustment: onset within 3 months
- Bulimia: weekly for 3 months
- Insomnia: 3 nights a week for 3 months
- SUD early remission: 3 months
- IED (verbal or nondamaging): twice weekly for 3 months

6 months

- Schizophrenia: 6 months total
- Prolonged grief (youth): 6 months since death
- PTSD delayed expression: 6 months or later
- Adjustment: ends within 6 months after stressor ends
- GAD: 6 months
- Illness anxiety: 6 months

12 months

- PDD or cyclothymia (youth): 1 year
- Rapid cycling: 4 episodes in 12 months
- Prolonged grief (adult): 12 months since death
- SUD sustained remission: 12 months
- SUD criteria window: 12 months
- Personality disorder under 18: 1 year (never ASPD)

2 years

- Persistent depressive disorder (adult): 2 years
- Cyclothymic disorder (adult): 2 years

Exam traps

- 'More than 1 month' (PTSD) is not the same as '1 month or more' (schizophreniform).
- Youth thresholds are shorter for persistent depressive disorder, cyclothymia, prolonged grief and separation anxiety disorder (4 weeks vs about 6 months).
- Adjustment disorder starts within 3 months of the stressor and ends within 6 months after it ends.

DSM-5-TR (APA, 2022), thresholds as summarized in this app's Timeline · Original study aid; criteria paraphrased.